Encounter overview

Patient and episode

Patient: Avery Testwell. Synthetic patient ID: CAP-TEST-001. Age: 42 years. Sex recorded: female. Encounter dates: 2026-09-14 to 2026-09-17. Report compiled: 2026-09-18. Facility: Fictional Capstone Training Hospital. All names, dates, observations, laboratory values, and reference intervals in this document are invented for software testing.

Reason for attendance

The fictional participant reported fatigue for approximately ten days, reduced appetite for three days, and occasional light-headedness when standing. The intake record reports no loss of consciousness. Symptoms were described by the participant and are not independently verified. The participant requested a review of recent laboratory results and an explanation of how the recorded results changed during the observation period.

History as recorded

The intake worksheet lists a history of hypertension. The worksheet does not provide a date of diagnosis or external evidence confirming this condition. No surgical history was entered. Family history was left blank. The smoking-history field reads NOT RECORDED. These absent details must not be filled using assumptions. The participant reported no known medication allergies, but the medication-reconciliation note says allergy verification was incomplete.

Episode boundaries

This document contains one fictional patient across several dated observations. Repeat tests belong to separate specimen events. A value from 2026-09-14 must not be substituted for a value from 2026-09-16. This document is a patient-report fixture, not a guideline document, and must not be indexed as recommendation guidance.

Intake observations

At 09:10 on 2026-09-14, the recorded seated blood pressure was 142/88 mmHg, pulse 92 beats/min, respiratory rate 18 breaths/min, temperature 37.1 degrees C, and oxygen saturation 98 percent on room air. The intake sheet supplies no numeric reference ranges for these observations. Weight was 68.0 kg. Height was NOT RECORDED. No interpretation should be inferred from these observations alone.

Initial complete blood count

Specimen event

Specimen ID: CBC-A-0914. Collection date: 2026-09-14. Collection time: 09:40. Material: venous blood. The worksheet records no collection problems. Reference intervals below are fictional, supplied solely for deterministic range-comparison tests. They are not clinical guidance.

Hemoglobin

Hemoglobin: 10.2 g/dL. Reference range: 12.0-16.0 g/dL. Source field: CBC-A-0914/HGB. No diagnostic interpretation appears in the source worksheet.

Hematocrit

Hematocrit: 32.0 %. Reference range: 36.0-46.0 %. Source field: CBC-A-0914/HCT.

White blood cell count

White blood cell count: 7.4 x10^9/L. Reference range: 4.0-11.0 x10^9/L. Source field: CBC-A-0914/WBC.

Platelet count

Platelet count: 420 x10^9/L. Reference range: 150-400 x10^9/L. Source field: CBC-A-0914/PLT.

Red cell indices

Mean corpuscular volume: 76 fL. Reference range: 80-100 fL. Mean corpuscular hemoglobin: 24 pg. Reference range: 27-33 pg. Red cell distribution width: 16.5 %. Reference range: 11.5-14.5 %.

Source limitations

The differential-count section was not supplied. The report does not include blood-film findings, prior-year results, or a confirmed cause for the observations. A report-analysis output should preserve the measurements and their supplied ranges without diagnosing a cause.

Initial chemistry and glucose

Specimen event

Specimen ID: CHEM-A-0914. Collection date: 2026-09-14 at 09:45. Material: serum. The fasting-status field reads UNKNOWN. Chemistry intervals in this fixture are fictional test data.

Electrolytes

Sodium: 138 mmol/L. Reference range: 135-145 mmol/L. Potassium: 3.3 mmol/L. Reference range: 3.5-5.1 mmol/L. Chloride: 102 mmol/L. Reference range: 98-107 mmol/L. Bicarbonate: 24 mmol/L. Reference range: 22-29 mmol/L.

Renal worksheet

Creatinine: 0.90 mg/dL. Reference range: 0.60-1.10 mg/dL. Blood urea nitrogen: 15 mg/dL. Reference range: 7-20 mg/dL. The estimated-filtration field was not reported. The analysis must not calculate or invent an estimated-filtration result.

Glucose worksheet

Glucose: 118 mg/dL. Reference range: 70-99 mg/dL. The supplied interval is recorded here exactly as printed. Fasting status is unknown. The numerical comparison and the uncertainty about applicability of the interval must remain separate. No diagnosis is supplied.

Protein and liver worksheet

Albumin: 4.1 g/dL. Reference range: 3.5-5.0 g/dL. Total protein: 7.2 g/dL. Reference range: 6.0-8.3 g/dL. ALT: 21 U/L. Reference range: 7-35 U/L. AST: 19 U/L. Reference range: 10-35 U/L.

No invented values

The laboratory worksheet contains no calcium, magnesium, bilirubin, or alkaline-phosphatase results. Missing analytes must not be generated from surrounding context. Test names, values, units, specimen date, and supplied intervals are distinct fields.

Supplementary tests and review cases

Specimen event

Supplementary worksheet SUP-A-0915. Collection date: 2026-09-15 at 08:30. All fields below belong to the same fictional patient. Several fields intentionally use formats outside the current deterministic parser scope.

Missing reference range

Ferritin: 8 ng/mL. Reference range: NOT PROVIDED. The measurement can be extracted, but the source does not support a below/within/above classification using a supplied interval. A review warning is expected.

Mismatched units

Total cholesterol: 5.8 mmol/L. Printed reference range: 125-200 mg/dL. Preserve both units exactly. Do not compare these numbers directly or silently convert the units. A review warning is expected.

Inequality range

C-reactive protein: 7 mg/L. Printed reference range: less than 5 mg/L. This is deliberately an inequality-format fixture. The current implementation lists inequalities as unsupported; preserve the source and flag review rather than forcing a closed interval.

Conditional range

Thyroid-stimulating hormone: 2.1 mIU/L. Printed reference text: adult interval 0.4-4.0 mIU/L; pregnancy-specific intervals differ. Pregnancy status: NOT RECORDED. This tests conditional reference handling, not a clinical decision.

Narrative-only entry

A urinalysis worksheet reads: specimen not received; no result available. There is no numeric urinalysis result. An analysis must not convert this narrative into a negative test.

Ambiguous interval

Vitamin B12: 260 pg/mL. Printed reference field: see age-specific worksheet. The referenced worksheet is absent. Preserve the value, mark the interval as unavailable or ambiguous, and do not invent an age-specific range.

Repeat laboratory measurements

Specimen event

Specimen ID: CBC-B-0916 and CHEM-B-0916. Collection date: 2026-09-16 at 08:15. These are repeat results, not corrections to the initial specimen. Supplied reference intervals are fictional.

Repeat blood count

Hemoglobin: 10.6 g/dL. Reference range: 12.0-16.0 g/dL. Hematocrit: 33.0 %. Reference range: 36.0-46.0 %. White blood cell count: 6.9 x10^9/L. Reference range: 4.0-11.0 x10^9/L. Platelet count: 395 x10^9/L. Reference range: 150-400 x10^9/L.

Repeat chemistry

Potassium: 3.8 mmol/L. Reference range: 3.5-5.1 mmol/L. Sodium: 140 mmol/L. Reference range: 135-145 mmol/L. Creatinine: 0.88 mg/dL. Reference range: 0.60-1.10 mg/dL. Glucose: 96 mg/dL. Reference range: 70-99 mg/dL. Repeat fasting status: recorded as fasting in the worksheet.

Chronological preservation

The initial potassium of 3.3 mmol/L appears on page 3 and the repeat potassium of 3.8 mmol/L appears on this page. Both observations should remain traceable to their own specimen event. The repeat platelet count of 395 x10^9/L does not erase the initial count of 420 x10^9/L.

No causal conclusion

The worksheet does not establish why any measurement changed. The report provides no transfusion record or medication-administration evidence explaining the trend. A summary may describe recorded changes but must not claim that a particular treatment caused them.

Boundary values

Training control result, not a clinical analyte: Range-check control A: 3.5 mmol/L. Reference range: 3.5-5.1 mmol/L. Range-check control B: 5.1 mmol/L. Reference range: 3.5-5.1 mmol/L. These exercise inclusive interval boundaries and must be labeled as training controls, not patient diagnoses.

Medication reconciliation and source provenance

Participant-reported history

At intake, the fictional participant reported taking a daily blood-pressure tablet but could not name it. The medicine name, dose, and frequency were recorded as UNKNOWN. No medication name may be inferred from the hypertension history.

Reconciliation worksheet

A later worksheet records: medicine label Training Tablet A; dose field NOT RECORDED; frequency field once daily; source participant statement. Training Tablet A is an invented label. This document supplies no prescription or administration record verifying that entry.

Allergy reconciliation

The intake note states no known medication allergies, source participant statement. The reconciliation form states allergy verification incomplete, source reviewer note. Both statements should be retained without turning incomplete verification into confirmed absence of allergies.

Encounter note

At 14:20 on 2026-09-16, the participant described less light-headedness than at intake. Fatigue remained present. The observer wrote that the participant was alert and able to answer questions. No formal neurological assessment, imaging finding, or specialist diagnosis was entered.

Administrative data

Observation area: training room 12. Recorded reviewer: Fictional Reviewer A. Insurer field: NOT RECORDED. Administrative labels are included for extraction realism; they are not evidence of clinical findings.

Provenance requirements

An extracted finding should cite an actual page and an excerpt present in that page. A medication statement attributed to the participant should not be relabeled as a clinician-confirmed prescription. Uncertainty and original source attribution should remain visible.

Observation notes and episode closure

Observation timeline

On 2026-09-14 at 16:00, the participant reported fatigue without a new complaint. On 2026-09-15 at 10:30, an observation note records that the participant drank from the practice intake cup; no volume was recorded. On 2026-09-16 at 14:20, the participant reported reduced light-headedness. These are narrative observations, not treatment recommendations.

Closing observations

On 2026-09-17 at 09:00, seated blood pressure was recorded as 134/82 mmHg, pulse 80 beats/min, respiratory rate 16 breaths/min, temperature 36.8 degrees C, and oxygen saturation 99 percent on room air. No explicit reference ranges accompany these measurements.

Episode status

The administrative closure field reads observation episode completed. The worksheet does not contain a signed discharge prescription, confirmed new diagnosis, or date for follow-up. An analysis or summary must not supply missing discharge instructions.

Unresolved source issues

The reference range is missing for ferritin; cholesterol units differ from those of its printed interval; an inequality is used for C-reactive protein; and a conditional interval appears for thyroid-stimulating hormone. Those items require review under the current software limitations.

What this report supports

This fixture supports testing page-linked extraction, numeric comparisons against supplied simple intervals, preservation of repeat results, and explicit handling of absent or ambiguous information. It does not establish medical accuracy or authorize care decisions.

What this report does not contain

No external guideline passages, imaging results, microbiology results, medication administration schedule, or confirmed cause of fatigue are present. Those details must not appear as established findings in generated analysis.

Record completeness and interpretation boundaries

Completeness checklist

Patient identity is consistent throughout: Avery Testwell, CAP-TEST-001. Report period is 2026-09-14 to 2026-09-17. Initial blood count and chemistry, supplementary tests, repeat tests, reconciliation, and closure are documented. Family history, height, verified prescription details, and follow-up date remain missing.

Source hierarchy

Numeric laboratory fields support extraction of the printed result and interval. Participant statements support reporting what the participant said, with attribution. Administrative closure supports only the recorded episode status. None of these sources supplies independent reference guidelines.

Date and page handling

Use ISO-formatted dates in this document to avoid day/month ambiguity. Page references must refer to physical PDF page numbers. Do not claim the compilation date is a specimen-collection date. The same analyte on different dates must retain separate evidence.

Review queue

Items with missing reference ranges, mismatched units, conditional intervals, or unsupported inequality formats should remain unclassified or explicitly flagged for review. A software warning is not a diagnosis and does not imply that a value is medically dangerous.

Summary constraints

A later summary should preserve the initial and repeat measurements where relevant, retain important uncertainties, and avoid inventing new diagnoses or prescriptions. Recommendation generation requires a separate reference-guideline source and human approval.

Fixture declaration

Every detail in this eight-page document is synthetic. Numeric reference intervals are test fixtures rather than authoritative medical standards. The file is designed for a single-patient capstone demonstration using selectable text. It contains no actual patient information. End of record.